

OpenEMR Billing & Configuration Guide

Version 3.0 · September 3, 2026

Source of truth for OpenEMR billing setup. Supersedes the billing and configuration content of GFC_OpenEMR_Deploy_Setup_Guide_v2.3 (July 22, 2026) and folds in GFC_OpenEMR_Billing_Setup_Checklist v5.

How to use this document

Scope. v2.3 Part I (the AWS deployment: security groups, RDS, EC2, DNS, Docker install, backups, cost alarm) is unchanged, already executed, and remains the deployment record. Keep that PDF. This document replaces everything from v2.3 Phase 7 onward and is the authority on identity, facilities, codes, billing, and compliance.

The structural change in v3. v2.3 was written around two named people and one facility. GFC now sees patients across multiple senior communities and private residences, with clinicians of differing specialties. So this version configures **archetypes** and treats the current roster as instances of them. Adding a fourth NP or a sixth facility becomes a form-fill against a defined pattern, not a redesign.

Section 12 is the care management line. CCM, CoCM, and BHI run at launch on an external platform integrated with OpenEMR. That section defines the split and the compliance gates.

Section 13 is the audit. Fourteen gaps found in v2.3, four of which create real financial or legal exposure. Read that section even if you read nothing else.

1. Current deployment state

Carried from v2.3, unchanged.

ITEM	VALUE
Application	OpenEMR 7.0.4, pinned, Docker on EC2
Host	EC2 t3.small, Ubuntu 24.04, 30 GB encrypted
Database	RDS MySQL 8.0, db.t4g.micro, encrypted, 30-day backups
Address	emr.godwinsfamilycarellc.com, Elastic IP
Region	us-east-1
Run cost	~\$40–55/month
AWS BAA	Accepted via AWS Artifact
Backups	Daily EBS snapshots, 14-day retention; RDS automated, 30-day

Do not deploy PHI on AWS Lightsail. It is not on the HIPAA-eligible services list. EC2 and RDS are.

2. Design principle: archetypes, not people

Every provider record, facility record, and permission set in OpenEMR is built from a numbered archetype below. Named individuals and named facilities are instances.

When you add a clinician, you pick an archetype, copy its configuration, and fill in that person's identifiers. When you add a facility, same. If a new person or place does not fit an existing archetype, define a new one here first, then build it. Never configure a one-off.

This is what keeps the claim reconciliation rule enforceable at scale:

For every billed line, GFC must show that enrollment status, rendering provider, certification and scope, EHR signature, service location and POS, eligibility, diagnosis support, and claim data all describe the same real-world service.

3. Provider archetypes

ID	ARCHETYPE	TAXONOMY	OPENEMR ACL	RENDERS?	MAY BILL
A1	Primary care NP	363LF0000X	Clinical	Yes	Home/residence E/M, AWV, ACP, TCM, CCM
A2	Dual-certified NP, primary + psych	363LF0000X and 363LP0808X	Clinical	Yes	A1 set, plus the psych set
A3	Specialty NP (gero, palliative, acute, wound)	Per certification	Clinical	Yes	Scope-limited subset, defined at onboarding
A4	Physician (MD/DO)	Per specialty	Clinical	Yes	Full scope; the only Schedule II prescriber
A5	1099 contract examiner (IME / C&P)	Per credential	Clinical, restricted	Yes	IME work only, not Medicare E/M
A6	Non-rendering owner / administrator	None	Administrators	No	Nothing. Ownership and managing control only
A7	Billing / coder	None	Accounting	No	Nothing. Charge review and claim work
A8	Front office / scheduling	None	Front Office	No	Nothing. Scheduling and demographics
A9	Clinical social worker (LCSW)	1041C0700X	Clinical, behavioral	Yes	Psychotherapy and assessment. No E/M
A10	Master's social worker (LMSW)	104100000X	Clinical, restricted	Not independently	Nothing directly. Bills only as a CoCM care manager under an NP

Current roster

PERSON	ARCHETYPE	TYPE 1 NPI	NOTES
Bethel Godwins	A1	1902310568	GA license RM186487. Primary employer is an East Point practice; GFC is an additional reassignment
Thanmayie Bethi	A2	1962038307	GA license 246736. NPPES currently lists only the FNP taxonomy. Psych billing is blocked until 363LP0808X is added
Bianca Ume	A6	None	Owner and managing director. Appears on the CMS-855B in ownership and managing control, never as a practitioner
Collaborating physician	A4	TBD	Required for both NPs' protocol agreements. Also the Schedule II referral path
Biller	A7	—	ACL group configured and empty until hired

Onboarding checklist per rendering clinician (A1–A5)

Run this for every clinician, every time. No exceptions, no partial builds.

- ☐ State license verified at the issuing board, dated screenshot retained
- ☐ National certification(s) verified with the issuing body, certificate copy retained
- ☐ Type 1 NPI confirmed in NPPES, with **every taxonomy they will bill under listed**, primary designated deliberately
- ☐ NPPES credential field and practice address accurate
- ☐ CMS-855I filed, with reassignment to GFC in the same application
- ☐ **OIG LEIE and SAM.gov exclusion screening cleared** (see §13.1)
- ☐ Malpractice coverage confirmed for every scope they will practice
- ☐ DEA registration and Georgia PDMP enrollment, if prescribing
- ☐ Nurse protocol agreement executed and filed with GCMB, if APRN
- ☐ Distinct OpenEMR user, Provider and Authorized flags, individual NPI, all taxonomies, own electronic signature, MFA enrolled
- ☐ Credential expiration dates entered in the tracker (§14)
- ☐ Code set they are permitted to bill recorded against their user record

No shared logins, ever. One human, one account. The login drives the note signature, which must match the claim.

3.1 Social work scope — A9 and A10

Social workers are a separate benefit category from nurse practitioners, with their own payment rate and a hard line at E/M. Getting this wrong in either direction costs money or creates liability, so it is spelled out.

A9 — LCSW. Medicare's "clinical social worker" under 42 CFR 410.73 requires a master's degree in social work, **at least 2 years or 3,000 hours of post-master's supervised clinical practice**, and state licensure to perform clinical social work. In Georgia that is the LCSW, not the LMSW.

- Enrolls independently: own Type 1 NPI, CMS-855I, reassignment to GFC, same onboarding checklist as any rendering clinician
- **Paid at 75% of the physician fee schedule.** For comparison: NP 85%, physician 100%
- **Cannot bill E/M codes.** CMS is explicit that E/M is built on a physical assessment, so 99341–99350 are unavailable to an LCSW no matter how long the visit

- Setting exclusions: inpatients of Medicare-participating hospitals and SNFs, and ESRD patients at dialysis facilities.
- Personal care homes, assisted living, group homes, and private residences are all fine**, so GFC's book is unaffected

A10 — LMSW. Not a Medicare-enrollable provider type. An LMSW cannot bill Medicare directly, and incident-to is not a workaround: incident-to requires direct supervision, meaning the billing practitioner physically present at the service location for the whole encounter. In a private residence that means the NP standing in the home while the LMSW works. That is not a business model, and behavioral health incident-to is a known audit target.

Three legitimate uses for an LMSW:

1. **Behavioral health care manager under Collaborative Care.** This is the one place LMSW time becomes Medicare revenue. The BHCM needs formal education or specialized training in behavioral health but **does not need independent licensure**. The NP is the billing practitioner for 99492, 99493, 99494, and 99484. Requires a psychiatric consultant, a patient registry with systematic tracking, documented patient consent, and monthly time tracking
2. **Private-pay care coordination.** GFC's coordination spine and liaison work, billed outside Medicare entirely
3. **Accruing supervised hours toward LCSW**, at which point they convert to A9

Flag before building CoCM: CMS defines the psychiatric consultant as a medical professional trained in psychiatry and qualified to prescribe the full range of medications. Georgia bars APRNs from Schedule II, so whether an A2 dual-certified NP satisfies "full range" is unsettled. Confirm with Palmetto in writing before staffing the consultant role with an NP rather than a psychiatrist.

3.2 Who may bill what

SERVICE	A1 NP	A2 NP DUAL	A4 PHYSICIAN	A9 LCSW	A10 LMSW
Home/residence E/M 99341–99350	Yes	Yes	Yes	No	No
AWV G0438 / G0439	Yes	Yes	Yes	No	No
ACP 99497	Yes	Yes	Yes	No	No
Psych diagnostic eval 90791	No	Yes	Yes	Yes	No
90792, with medical services	No	Yes	Yes	No	No
Psychotherapy 90832 / 90834 / 90837	No	Yes	Yes	Yes	No
Psychotherapy add-on to E/M +90833 etc.	No	Yes	Yes	No	No
Family psychotherapy 90846 / 90847	No	Yes	Yes	Yes	No
CCM 99490 / 99439 / 99487 / 99489	Billing practitioner	Billing practitioner	Billing practitioner	Clinical staff time	Clinical staff time
CCM practitioner-only 99491 / 99437	Yes	Yes	Yes	No	No
CoCM 99492–99494, BHI 99484	Billing practitioner	Billing practitioner	Billing practitioner	Care manager	Care manager
Schedule II prescribing	No	No	Yes	No	No
Payment rate	85%	85%	100%	75%	n/a

This table is the source for the certification-to-code check in the charge-ready gate (\$10).

4. Facility archetypes and place of service

POS describes where the service physically happened. It is not a preference and not a facility-wide default. GFC sees patients in several setting types, so OpenEMR carries one facility record per physical site, each with its own POS, and the encounter can override.

ID	SETTING	POS	E/M CODE FAMILY
F1	Private residence, patient's own home	12	99341–99350
F2	Assisted living community	13	99341–99350
F3	Group home	14	99341–99350
F4	Custodial care facility	33	99341–99350
F5	Skilled nursing facility	31	99304–99310, not the home family
F6	Nursing facility, non-skilled	32	99304–99310
F7	Telehealth, patient at home	10	Per service, plus modifier
F8	Telehealth, patient elsewhere	02	Per service, plus modifier

The Georgia wrinkle. Georgia licenses personal care homes and assisted living communities as separate categories, and neither label maps one-to-one onto a CMS POS. POS follows the setting's actual characteristics, not the state's license name. POS 13 describes a congregate residential facility with self-contained living units, resident needs assessment, and 24/7 on-site support. POS 14 describes a residence with shared living areas where residents receive supervision plus custodial and minimal services.

Method for every new facility:

1. Obtain the DCH license type, number, and bed count
2. Record the actual operating model: unit structure, staffing coverage, services delivered or arranged
3. Map to F1–F8 above
4. If borderline, request written guidance from Palmetto GBA and file the response with the facility record
5. Build the OpenEMR facility record with that POS
6. Confirm no other entity bills an overlapping professional or facility service for those residents

Do not default to POS 12 because the code family is called "home or residence." That was the v2.3 error.

Current facility instances

FACILITY	ARCHETYPE	POS	STATUS
Hickory Log Personal Care Homes	F2 or F3, pending	TBD	Blocked on DCH license verification
Private-residence clients (Ellijay and others)	F1	12	Active
Telehealth, behavioral health	F7	10	Pending psych go-live

Per-facility record requirements

- Legal name, street address, unit, phone, administrator contact
- Correct POS
- Facility policy on outside practitioners, medication orders, resident consent, records access, emergency escalation

- BAA or equivalent agreement where GFC handles the facility's PHI
- Marked as a **service location**. GFC remains the billing location

5. Entity, NPI, and enrollment

Entity decision

All service lines stay under GFC LLC. Medicare filings go under that EIN. The split into separate entities is deferred, not cancelled.

LINE	WHAT IT IS	REGULATION	PAYMENT
Private home care	Non-medical personal care, ADLs, companionship	GA DCH Private Home Care Provider license	Private pay. Never touches OpenEMR billing
In-home primary care	A medical practice seeing patients where they live	Clinician licensure	Medicare Part B
In-home psych	Behavioral health in the residence	Clinician licensure and certification	Medicare Part B

None of these is Medicare home health, which is a certified agency delivering skilled nursing and therapy under a physician plan of care with a homebound requirement. GFC is not that and should stop signalling it.

What this decision accepts. Medicare enrollment attaches to the EIN. If the clinical lines later move to a separate entity, enrollment restarts and billing stops during the gap, and that cost grows with volume. Psych liability also reaches the home care assets. Mitigations: separate books and bank accounts by line, separate insurance policies, documented cost allocation, and a **written trigger** for revisiting the split rather than leaving it open-ended.

NPI structure

An organization obtains separate NPIs for subparts that would be separate covered providers if they stood alone. Subparts share the EIN, so this works under one roof.

NPI	LINE	TAXONOMY	USE
1710678339 (existing)	Home care	Correct to 253Z00000X In Home Supportive Care, or 251J00000X, matched to the DCH license	Private pay. No Medicare claims
New Type 2	Clinical practice	193200000X Multi-Specialty Group	Billing entity on every Part B claim

The existing NPI is currently tagged 251E00000X Home Health, which describes a Medicare-certified agency and fits neither line. Correct it.

Address conflict to resolve first. NPPES has GFC at 4945 Baker Plantation Way, Acworth. v2.3 §7.3 built the facility record at 4300 Paces Ferry Road SE, Suite 500, Atlanta. Pick one, then make NPPES, PECOS, the 855B, the IRS record, and OpenEMR all agree. Address mismatch is the most common cause of a stalled enrollment.

Enrollment

The entity. Reassignment only works if the organization holds an active Part B group enrollment. An NPI is an identifier, not an enrollment.

- File the **CMS-855B**, revision 04/29/2026. That version has been the only one accepted since 08/03/2026; older versions are returned and add 30 to 60 days
- Reassignment is reported in Section 4H, practitioner signature in Section 15E
- **No application fee.** 42 CFR 424.502 defines "institutional provider" as one submitting the CMS-855B "(not including physician and nonphysician practitioner organizations)." A nurse practitioner organization is carved out
- A6 appears in ownership and managing control

Practitioners. A clinician may reassign benefits to more than one organization simultaneously. Each reassignment is filed separately, one entity per application. An outside primary employer stays primary; GFC is an additional reassignment.

What keeps it clean: work billed under GFC must be work performed for GFC, at GFC's service location, in GFC's chart, on GFC's schedule.

The exposure is contractual, not regulatory. **Check any clinician's outside employment agreement for exclusivity or outside-activity clauses before filing anything naming them.**

The CMS-855R is retired. Merged into the CMS-855I effective 09/01/2023; MACs have rejected the standalone form since 11/01/2023. Reassignment runs through PECOS or the current 855I. Scrub any GFC document still referencing it.

Retrospective billing

The effective date of billing privileges is the later of the filing date of a subsequently approved application or the date services first began at the location. Retrospective billing may be permitted up to 30 days before that effective date, and only where the supplier met all program requirements, including licensure, during that window.

Operating instruction: document every visit completely, and hold any claim whose rendering provider's enrollment or reassignment is unresolved for that date of service. Timely filing runs 12 months for Medicare. Holding a claim costs nothing. Releasing one under a broken reassignment costs a great deal.

6. Georgia scope and nurse protocol agreements

Applies to every APRN archetype (A1, A2, A3). Georgia updated these rules effective 05/25/2026.

- A delegating physician may cover **up to the combined equivalent of eight APRNs or PAs at any one time**
- Agreements filed with GCMB **within 30 days of execution**, amendments on the same timeline
- An incomplete application goes invalid after three months and must be resubmitted from scratch
- On termination: physician notifies the Board within 10 working days, APRN within 7 days
- Chart review by the delegating physician: **100% of records involving controlled substance prescriptions, at least quarterly; 100% of records with adverse outcomes within 30 days of discovery; 10% of all remaining records annually**

That chart review is real recurring work. Price it into the collaborating physician agreement rather than assuming it as a favor.

The Schedule II constraint

Georgia APRNs remain prohibited from prescribing Schedule I and Schedule II substances, with one narrow exception: hydrocodone, oxycodone, or compounds thereof, in emergency situations only, initial prescription capped at a 5-day supply,

patient 18 or older, authority explicitly in the protocol agreement, one year post-licensure experience, biennial CE, and DEA registration updated accordingly.

Consequence for the psych line: stimulants are Schedule II. No APRN archetype can prescribe Adderall, Ritalin, Vyvanse, or equivalents in Georgia. Antidepressants, most antipsychotics, and mood stabilizers are unscheduled and unaffected. Benzodiazepines are Schedule IV and available under protocol.

☐ Written Schedule II referral pathway: which A4 physician, or which outside prescriber, and how the handoff is documented

7. OpenEMR identity and practice build

Replaces v2.3 §7.1–7.5.

7.1 Global security

Administration → Config:

- **Security:** idle session timeout 900 seconds; password expiration 90 days; minimum length 12; strong passwords required; lockout after 5 failed logins
- **Logging:** audit logging on, with every patient-record subcategory enabled including views, updates, and disclosures. No truncation of PHI-access events
- **Appearance:** application title "Godwins Family Care"
- Save on each tab. Settings do not persist tab-to-tab without it

MFA on every account, including A6 and A7. The admin account first.

7.2 Organization record

- Name: Godwins Family Care LLC. Never "Godwins" alone
- Address: the one resolved in §5
- Tax ID: GFC LLC's EIN
- Organizational NPI: **the new clinical Type 2 NPI**, once enrollment lands. Blank until then
- Billing Location, Accepts Assignment, Service Location all checked

v2.3 instructed leaving the organizational NPI blank permanently and billing under an individual's NPI. That reflected an entity structure that has since been decided. Populate this field when the 855B is approved. It is one field, and it redirects payment from an individual to the business.

7.3 Facility records

One record per physical site from §4, each with its own POS. Hickory Log and every future community gets its own. Private-residence clients use the F1 record.

7.4 Provider records

Build from the §3 archetype table. Every rendering clinician: Provider and Authorized checked, individual NPI, **all applicable taxonomies**, state license, DEA where relevant, MFA.

7.5 Access control — corrected

v2.3 §7.5 assigned the Clinical group "no Fees/Financial permissions," then its own acceptance test had a clinician adding a CPT code to a Fee Sheet. Those contradict, and the restrictive reading breaks same-day charge capture, which is the single highest-leverage habit in a small practice.

Resolution: clinicians get Fee Sheet write, not financial visibility.

ARCHETYPE	ACL GROUP	FEE SHEET	PAYMENTS / AR / ADJUSTMENTS	REPORTS
A1–A5	Clinical	Write	No	Own encounters only
A6	Administrators	Write	Yes	All
A7	Accounting	Write	Yes	Financial
A8	Front Office	No	No	Scheduling only

Clients, families, and caregivers never log in to OpenEMR. The native patient portal stays disabled; the GFC app is the only patient-facing surface.

8. Codes, modifiers, and the fee schedule

OpenEMR ships with no CPT codes. Enter only what GFC bills, at Administration → Coding → Codes.

8.1 Code sets by archetype

A1 and A2, primary care

- Home/residence E/M, new patient: 99341, 99342, 99344, 99345. **99343 was deleted; do not enter it**
- Home/residence E/M, established: 99347, 99348, 99349, 99350
- Nursing facility E/M, if F5 or F6 is ever served: 99304–99310. Different family, different rules
- G0438 initial AWW, G0439 subsequent AWW
- 99497 advance care planning
- 99495, 99496 transitional care management
- CCM, CoCM, and BHI: live at launch on an external platform. See §12

A2 only, psych

CODE	SERVICE
90791	Psychiatric diagnostic evaluation, no medical services
90792	Same, with medical services. Requires a prescriber
90832 / 90834 / 90837	Psychotherapy, 30 / 45 / 60 minutes
+90833 / +90836 / +90838	Psychotherapy add-on performed with an E/M
96127	Brief emotional or behavioral assessment
96116	Neurobehavioral status exam

A9, clinical social worker

- 90791 psychiatric diagnostic evaluation. **Not 90792**, which requires medical services
- 90832 / 90834 / 90837 psychotherapy
- 90846 / 90847 family psychotherapy, with and without the patient
- 90853 group psychotherapy
- 96127 brief emotional or behavioral assessment
- Health behavior assessment and intervention codes, where applicable
- **No E/M codes under any circumstance**

Care management, billed by A1, A2, or A4. Full code set, rules, and platform architecture in §12.

Billing rules that matter:

- The outpatient mental health treatment limitation is long gone. Psych carries standard Part B cost sharing, the same 20% as medical
- When psychotherapy and an E/M happen in one encounter, bill the E/M plus the timed add-on, not a standalone psychotherapy code
- Psychotherapy time is documented separately from E/M time. The same minutes cannot be counted twice
- If any substance use disorder treatment records are held, 42 CFR Part 2 applies on top of HIPAA with its own consent handling

8.2 Modifiers — missing entirely from v2.3

Configure these before the first claim. Without them, legitimate same-day services get bundled and denied.

MODIFIER	WHEN
25	Significant, separately identifiable E/M on the same day as another service. The AWW plus a problem visit is the routine case, and without modifier 25 the E/M denies
95	Synchronous audio-video telehealth
93	Audio-only telehealth, where permitted for behavioral health
59 / X{EPSU}	Distinct procedural service. Use sparingly and only with documentation
GT	Legacy telehealth on some payers. Confirm per payer

8.3 Fee schedule — corrected

v2.3 §9.3 said to anchor fees to the current Medicare Physician Fee Schedule, Georgia locality. **Do not set charges equal to the Medicare allowable.** A payer never pays more than the billed charge, so charging at Medicare rates caps every commercial and Medicare Advantage payment at Medicare rates, permanently, even where the contract would have paid more.

Set the charge master at 125 to 150% of the Medicare allowable and take the contractual writedown at posting. Use the Medicare PFS Georgia locality as the reference point, not the charge.

- ☐ Enable the ICD-10 code set for fee sheet diagnosis selection
- ☐ Mark every code active and confirm it appears on a test encounter's Fee Sheet
- ☐ **Recurring January task: reload the fee schedule after the annual PFS update (§13)**

9. Per-patient setup

- Demographics: name exactly as on the Medicare card, DOB, the residence address
- Insurance: MBI primary, Medigap or Medicaid secondary, subscriber equals self
- Signature on file YES for both release of information and assignment of benefits, backed by the intake packet signed by the patient or a legal representative. **A facility nurse supervisor cannot sign unless she holds documented legal authority**
- POA or responsible party recorded, with a **separate billing contact field**
- Existing PCP recorded
- Facility record assigned from §4, which drives POS

Keep these as **separate fields**, since each drives different billing behavior:

- Medicare Advantage enrollment and plan name
- Medicaid eligibility and category
- QMB status
- Secondary or Medigap carrier

Dual eligible and QMB are not the same. Every QMB is dual eligible; not every dual is QMB.

Two revenue assumptions that need verification per patient

Annual wellness visits. G0438 is generally once per lifetime, G0439 once per 12 months. Payment follows the beneficiary's claims history, not which practice considers itself primary. If an existing PCP has been doing them, the claim denies. Build a pre-check of last G0402, G0438, and G0439 dates from the Palmetto eServices portal. A 270/271 alone will not reliably return AWV history. Keep AWV out of any launch revenue forecast until the roster is checked.

Medicare Advantage. MA determines whether a visit is billable at all. Sort every roster into: Original Medicare FFS; MA in-network; MA out-of-network but potentially payable; MA requiring authorization or a single-case agreement; unverifiable. For an out-of-network MA patient, a claim to Palmetto is not payable. MA behavioral health often carries separate network and authorization rules from medical, so check both lanes.

10. Visit workflow and charge capture

Encounter defaults: service facility per §4, billing facility GFC, rendering provider set to the person in the room. Defaults must be overridable.

Each note carries:

- Service date, patient location, rendering clinician
- Medically appropriate history and exam
- Either the MDM basis or a total practitioner time statement. Psych encounters document psychotherapy time separately from E/M time
- Assessment and plan tied to each diagnosis used on the claim
- Medication reconciliation and a record of where prescribing was routed

- The rendering practitioner's own signature and attestation
- A late-entry policy preserving true date, time, and amendment reason

Prescribing. Route medication changes to the patient's established authorized prescriber until prescriptive authority, the GCMB protocol agreement, e-prescribing and EPCS setup, formulary, and the facility's order process are all confirmed for that clinician. Document the recommendation, the communication, the acceptance or rejection, and who ultimately prescribed. Any Schedule II medication routes out under §6 regardless of enrollment status.

The charge-ready gate

Nothing enters the billing queue until all eight are true:

1. Note signed and locked
2. Diagnosis-to-service linkage reviewed
3. Rendering provider validated against the §3 roster
4. **Rendering provider's certification matches the code billed**, checked against the §3.2 table. A1 cannot bill the psych set. A9 cannot bill E/M. A10 cannot bill anything independently
5. **Facility record and POS match where the service actually happened**
6. Eligibility and payer order validated
7. Enrollment and reassignment confirmed active for that date of service
8. Required consent, notice, or authorization documented

11. Claims, remittance, and patient responsibility

11.1 Payers and clearinghouse

- **Availity** is the X12 partner, version 005010X222A1 for 837P. Already configured in v2.3 §9.2 and in enrollment since July. Do not switch
- Payer IDs come from Availity's payer list, not the payer's website
- Priority order for the current book: Medicare Part B (Palmetto GBA, Jurisdiction J), then each Medicare Advantage plan on the roster as its own payer, then Medigap carriers, then Georgia Medicaid. Commercial payers are added only when a real patient carries them **and** GFC is contracted and credentialed with that payer (§13.3)
- EDI enrollments through Availity with Palmetto: 837 claims, 835 ERA, 270/271 eligibility
- Register Availity under GFC's group NPI and EIN once the 855B is approved, not under an individual

11.2 Claim flow

- Run 270/271 eligibility **before the visit and again before claim release**. Enrollment changes between those points
- Generate the 837 batch in Fees → Billing Manager, download, upload in Availity Essentials. Manual at launch volume, roughly five minutes per batch
- Send one test claim, confirm acceptance, then release the rest
- Validate payer IDs, provider IDs, facility and POS mapping, and the rendering field on the test before the batch goes
- **ERA auto-posting stays deferred**. Post manually for the first months to learn each payer's denial patterns

- Keep three failure modes separate: clearinghouse rejections, payer front-end rejections, and adjudicated denials. Different owners, different fixes
- Weekly denial and rejection report, worked every Friday

11.3 Patient responsibility

2026 figures: \$283 annual Part B deductible, then Medicare pays 80% of the approved amount and 20% is patient responsibility. Psych is treated the same as medical.

The 20% is of Medicare's allowed amount, not the charge. With a charge master at 125 to 150% of allowable, calculating patient responsibility by hand overcharges every patient. The number comes from the 835 remittance, which itemizes allowed amount, paid amount, deductible applied, coinsurance, and contractual adjustment.

Hard rule: no statement goes out before the ERA posts.

PATIENT SITUATION	WHO PAYS THE 20%	STATEMENT?
Medigap	Medigap, via automatic crossover	No
QMB	Nobody bills the patient. Write off what Medicaid does not pay	Never
Medicaid, non-QMB	Medicaid up to its allowable	No
Traditional Medicare, no secondary	Patient or POA	Yes
Medicare Advantage	Plan copay, per the plan's EOB	Per plan

No cost sharing at all on AWWs (G0438, G0439), or on ACP (99497) when furnished with a covered AWW.

- ☐ **QMB hard hold** in OpenEMR blocking statements, collection tasks, and manual balance transfers for confirmed QMB patients. Federal law prohibits billing QMB individuals for Medicare-covered deductibles, coinsurance, or copays, including under Medicare Advantage, and including when Medicaid pays less than the full amount. This is a system control, not a staff instruction
- ☐ Statement trigger fires only on posted-ERA patient responsibility
- ☐ Medigap crossover confirmed on the first remittance rather than assumed; drop secondaries manually if it does not

11.4 Telehealth

Behavioral health telehealth is a real channel with a dated window.

- Medicare telehealth flexibilities extended through **12/31/2027**
- The patient's home remains an acceptable originating site for behavioral health through that period, with no in-person visit requirement
- From **01/01/2028**, an in-person visit within six months prior to the first behavioral telehealth service, and annually thereafter. Most specialties lose the home as an originating site; behavioral health keeps it
- **Patients established on behavioral telehealth on or before 12/31/2027 are grandfathered** past the initial six-month requirement

Front-load establishment dates deliberately and track them per patient. Confirm with Palmetto whether a resident in a licensed community counts as "home" for POS 10 purposes before billing that way.

12. Care management service line

Live at launch. CCM, Collaborative Care, and BHI run on an external care management platform, integrated with OpenEMR. This section defines the split, the codes, and the gates.

12.1 The architecture

SYSTEM	ROLE
External platform	Registry, comprehensive care plan, time tracking, outreach logging, task queues. The operational system the care team works in daily
OpenEMR	Billing system of record. Every charge is entered here, because the 837 is generated here and transmitted through Availity

The charge must land in OpenEMR. A care management platform that bills independently would split the claim stream and break reconciliation, denial tracking, and the ERA posting workflow.

Outbound, OpenEMR → platform. Patient roster, demographics, insurance and payer flags, problem list, medication list, allergies, encounter history. These are FHIR reads through the existing OAuth2 client: Patient, Condition, MedicationRequest, AllergyIntolerance, Observation. OpenEMR's FHIR lane is its strongest surface, so this direction is straightforward.

Inbound, platform → OpenEMR. Each month the platform must deliver, per patient: the CPT code earned, the ICD-10 codes supporting it, the service date, the rendering practitioner, and the total qualifying minutes. Plus the care plan document and the time-log summary, filed to the chart. Encounter and document writes ride the Standard REST lane per v2.3 Appendix D, not FHIR.

The time log stays in the platform but must be exportable and retained. In a CCM or CoCM audit the time log is the evidence. If the platform cannot produce a defensible per-patient, per-activity, per-minute record on demand, it is the wrong platform.

- ☐ BAA executed with the platform vendor before any PHI moves
- ☐ Platform vendor's own subcontractors and hosting reviewed
- ☐ Integration lanes verified against the live OpenEMR server, per v2.3 Appendix D method
- ☐ **Monthly reconciliation:** count of platform-eligible patients against charges posted in OpenEMR. A gap in either direction is a revenue leak or a compliance problem
- ☐ Time-log export tested and retained on GFC-controlled storage, not only in the vendor's system

12.2 Codes

Chronic care management

CODE	WHOSE TIME	THRESHOLD
99490	Clinical staff	First 20 minutes
+99439	Clinical staff	Each additional 20 minutes
99491	Practitioner only	First 30 minutes
+99437	Practitioner only	Each additional 30 minutes
99487	Clinical staff, complex	First 60 minutes
+99489	Clinical staff, complex	Each additional 30 minutes
G3002 / +G3003	Practitioner	Chronic pain management

The clinical staff codes (99490, 99439, 99487, 99489) include both staff and practitioner time. The practitioner codes (99491, 99437) count only the practitioner's own minutes. Do not mix them for the same patient in the same month.

Behavioral health

CODE	THRESHOLD
99492	CoCM, initial month, first 70 minutes
99493	CoCM, subsequent month, first 60 minutes
+99494	CoCM, each additional 30 minutes
99484	General BHI, 20 minutes

12.3 Enrollment gates, per patient

Every one of these must be true before a patient is enrolled. The platform should enforce them; OpenEMR carries the evidence.

1. **Two or more chronic conditions** expected to last at least 12 months or until death, placing the patient at significant risk of serious complication, decline, or death
2. **Initiating visit** completed: a comprehensive E/M, an AWW, or an IPPE, required for new patients or any patient not seen within the prior 12 months. The CCM discussion must happen inside that visit
3. **Consent** obtained and documented, written or verbal, covering: that the service is available; that **cost sharing applies**; that **only one practitioner may bill CCM for them in any calendar month**; and that they may stop at the end of any month. Record accepted or declined either way
4. **No other practitioner is already billing CCM** for that patient. Verify before enrolling, not after the denial
5. **Comprehensive electronic care plan** created and available promptly both inside and outside the practice
6. **24/7 access** in place

The launch synergy worth planning around: the Hickory Log visits are the initiating visits. The consent conversation belongs in the 9/4 visit script, not in a follow-up call weeks later. Build the consent capture into the encounter template now.

The 24/7 requirement is an operational commitment, not a checkbox. Patients must be able to reach a practitioner or clinical staff at any hour for urgent needs, with a designated care team member and more than one contact method. Decide who answers at 2am before the first patient enrolls.

Cost sharing is a patient conversation. CCM and CoCM carry the standard 20% coinsurance, which is a recurring monthly charge. QMB patients are billed nothing, so most of the Hickory Log roster is unaffected. For a traditional-Medicare patient with no secondary, it is a real monthly bill and a real churn risk. Disclose it plainly at consent.

12.4 Concurrency rules

- **One practitioner per patient per calendar month for CCM.** This is the rule most likely to bite in a facility population where residents already have a PCP
- **CCM and CoCM or BHI may be billed in the same month**, but no minute counts twice. Every logged activity belongs to exactly one program. A medication review counted toward CCM cannot also satisfy BHI
- Complex and non-complex CCM cannot both be billed in the same month
- CCM cannot be billed alongside home health or hospice supervision, or ESRD services 90951–90970
- Complex CCM cannot be billed with prolonged E/M in the same month

The platform must enforce program separation at the activity level. If it logs time to a patient rather than to a patient-plus-program, it cannot support concurrent billing safely.

12.5 Who does the work

- **CCM clinical staff time** runs under **general supervision**: the billing practitioner directs and controls but need not be present. This is what makes a remote care management team viable, and it is why an A10 LMSW's time is billable here
- **CoCM behavioral health care manager** may be A9 or A10
- **99491 and 99437** count only the practitioner's own time, so A1, A2, or A4 exclusively
- The billing practitioner on every care management claim is A1, A2, or A4. Never A9 or A10

13. Blind spots in v2.3

The audit. Fourteen gaps, ordered by exposure.

13.1 No exclusion screening — highest exposure

Nothing in v2.3 screens clinicians or staff against the OIG List of Excluded Individuals and Entities or SAM.gov. Billing Medicare for services furnished by an excluded individual creates false claims exposure, and the obligation to screen is ongoing, not one-time.

- ☐ Screen every clinician, employee, and contractor against OIG LEIE and SAM at hire
- ☐ Re-screen the full roster monthly. Retain dated results
- ☐ Add screening to the §3 onboarding checklist as a hard gate

13.2 No credit balance or overpayment process

Medicare overpayments must be reported and returned within 60 days of identification. v2.3 has no process for identifying credit balances, no owner, and no clock.

- ☐ Monthly credit balance report from OpenEMR
- ☐ Written procedure: identify, investigate, refund or report within 60 days
- ☐ Named owner

13.3 Commercial payers listed without a contracting path

v2.3 §9.1 instructs entering BCBS, Aetna, UnitedHealthcare, Cigna, and Humana as payers. Adding a payer to OpenEMR does not make you able to bill it. Each requires its own contract and credentialing, and commercial credentialing runs through **CAQH ProView**, which v2.3 never mentions.

- ☐ Do not enter a commercial payer until GFC holds a contract with it
- ☐ Build a CAQH ProView profile per clinician, kept current, before any commercial credentialing
- ☐ Track credentialing status per payer per clinician

13.4 Charge master set at Medicare rates

Covered in §8.3. Charging at the allowable permanently caps collections from every payer that would have paid more. Corrected to 125–150%.

13.5 Clinical ACL contradicts the charge capture workflow

Covered in §7.5. v2.3 denied clinicians fee permissions, then had a clinician complete a Fee Sheet in its own acceptance test. Resolved: Fee Sheet write, no financial visibility.

13.6 POS hardcoded to 12 at the facility level

Covered in §4. Broke the moment GFC saw a patient in a licensed community.

13.7 No modifiers configured

Covered in §8.2. Modifier 25 alone is worth real money on AWV-plus-problem visits.

13.8 No eligibility workflow

v2.3 enables 270/271 through Availity but never says to run it. Covered in §11.2, twice per claim.

13.9 No credential expiration tracking

Licenses, certifications, DEA, CAQH attestation, malpractice, and protocol agreements all expire. A lapsed credential mid-service-period makes every claim in that window suspect. Covered in §13.

13.10 No ABN process

When a service may not be covered, an Advance Beneficiary Notice is what makes the patient responsible. Without one, GFC absorbs the denial.

- ☐ ABN form stocked, staff trained on when to issue, signed copies filed to the chart

13.11 No write-off, adjustment, or refund policy

Where leakage and fraud live. v2.3 has no thresholds and no approval path.

- ☐ Written policy: who may adjust, at what dollar threshold, with what documentation
- ☐ Monthly adjustment report reviewed by someone other than the person who posted it

13.12 No segregation of duties, and no compensating control

A6 currently holds full clinical, billing, and administrative access, and is also the owner. At this size that is unavoidable. It should still be acknowledged with a compensating control.

- ☐ Monthly review of adjustments, write-offs, and refunds by a second person, logged
- ☐ Quarterly audit log review with findings written down

13.13 No BAA with Availity

v2.3 correctly requires the AWS BAA. It does not require one with the clearinghouse, which handles every claim's worth of PHI.

- ☐ BAA executed with Availity
- ☐ BAA with each facility where GFC handles their PHI
- ☐ BAA with the telehealth platform before psych go-live

13.14 Audit log review has no owner or standard

"Glance at the audit log monthly" is not a control. Covered in §13.

Also add

- Medicare **revalidation every five years**, with PECOS notices going to whatever contact is on file. Verify that contact is monitored
- **Timely filing** varies: 12 months for Medicare, often 90 to 180 days commercial. Track per payer
- **Record retention** schedule reconciling Georgia requirements with Medicare's
- **42 CFR Part 2** analysis before any SUD records are held
- Confirm OpenEMR **7.0.4 is still receiving security patches**. The v2.3 decision to defer 8.0 set a review for roughly late January 2027

14. Compliance calendar

Put these on a recurring schedule with a named owner. This is the part that decays silently.

CADENCE	TASK
Weekly	Denial and rejection report worked
Monthly	OIG LEIE and SAM re-screen, full roster
Monthly	Credit balance report; refunds inside the 60-day clock
Monthly	Adjustment and write-off review by a second person
Monthly	Server patch, OpenEMR patch, backup verification, cost check
Quarterly	Delegating physician chart review per §6, documented
Quarterly	Backup restore drill to a scratch instance
Quarterly	Audit log review with written findings
Annually	Credential expiration sweep: licenses, certifications, DEA, PDMP, CAQH attestation, malpractice, protocol agreements
Annually (January)	Reload the fee schedule after the Medicare PFS update
Annually	Facility POS re-verification against current DCH licenses
Every 5 years	Medicare revalidation

15. Deferred, deliberately

ITEM	WHY IT WAITS	TRIGGER
ERA auto-posting	Manual posting first teaches payer denial behavior	After several clean months
Availity SFTP and cron	Manual upload is five minutes per batch at launch volume	Volume
Advanced Primary Care Management (APCM)	A newer alternative to CCM with different structure and no time threshold. Evaluate against the CCM build once that is running	After the first CCM quarter
RPM codes	Out of scope	Track C
A7 billing account	ACL group configured and empty	Biller hired
System-to-system OAuth2 client	No background job exists yet	First background job
OpenEMR 8.0 upgrade	7.0.4 is patched and the menu paths are written against it	Review ~January 2027
Multi-AZ database, larger instance	Both are sub-10-minute upgrades that lose no data	Volume
Entity split	See §5	Written trigger, to be set

16. Open items

Blocking claim release

- ☐ Add psych taxonomy 363LP0808X to NPI 1962038307 in NPPES
- ☐ Resolve the Acworth vs Paces Ferry address conflict

- ☐ PECOS status for the group and each rendering clinician
- ☐ Correct NPPES taxonomy on 1710678339; obtain the clinical Type 2 NPI
- ☐ File the CMS-855B under GFC LLC; file reassignments through PECOS
- ☐ Hickory Log DCH license type and number; POS determination
- ☐ OIG and SAM screening for the full roster

Blocking clinical operations

- ☐ Outside employment agreement exclusivity check for any clinician with another employer
- ☐ Face sheets, MBIs, payer order, POA billing contacts, QMB flags, MA flags, PCP information
- ☐ Delegating physician identified, with scope covering every archetype's practice and capacity for the chart review load
- ☐ Schedule II referral pathway documented
- ☐ Home/residence E/M note template live
- ☐ Charge capture log identifying rendering clinician and date of service

Blocking care management go-live (§12)

- ☐ Care management platform selected; BAA executed before any PHI moves
- ☐ Integration lanes built and verified against the live OpenEMR server, both directions
- ☐ Time-log export tested and retained on GFC-controlled storage
- ☐ **Consent language built into the 9/4 encounter template**, so the launch visits double as initiating visits
- ☐ Per-patient check that no other practitioner is already billing CCM
- ☐ 24/7 access arrangement decided and staffed, with a named designated care team member
- ☐ Program-level time separation confirmed in the platform, so CCM and BHI minutes never overlap
- ☐ Monthly reconciliation report: platform-eligible patients versus charges posted in OpenEMR
- ☐ Cost-sharing disclosure script for non-QMB patients

Blocking psych go-live

- ☐ Both certifications verified for the A2 clinician
- ☐ CMS-855I and reassignment approved
- ☐ GCMB protocol agreement filed with psych scope explicit
- ☐ DEA and Georgia PDMP active
- ☐ Psych codes and note templates enabled
- ☐ Telehealth platform, BAA, and consent workflow, if telehealth is in scope

Structural and compliance

- ☐ Per-patient AWW history pull from Palmetto eServices
- ☐ MA roster split with network status and claim routing, medical and behavioral separately
- ☐ Separate books, bank accounts, and insurance policies by service line
- ☐ CM&F COI scope verified for behavioral health rendered by an NP, limits sized for psych
- ☐ Written trigger for revisiting the entity split
- ☐ Georgia healthcare attorney call on professional entity ownership

- ☐ For any social worker on staff: confirm LCSW vs LMSW, and whether the 3,000-hour post-master's requirement is met. That single fact decides A9 or A10
 - ☐ Written Palmetto confirmation on whether a PMHNP satisfies the CoCM psychiatric consultant definition, given Georgia's Schedule II bar
 - ☐ BAAs: Availity, each facility, telehealth platform
 - ☐ Write-off, adjustment, and refund policy
 - ☐ ABN process
 - ☐ Credential expiration tracker populated
 - ☐ Any GFC document still referencing a standalone CMS-855R
 - ☐ MFA rollout complete; tested restore performed
-

Appendix A — Acceptance test

Run before the first real claim. All must pass.

1. Log in as each rendering clinician with MFA. Calendar shows their availability and visit categories
2. Open a TEST patient assigned to each facility archetype in use. Confirm the encounter picks up the correct POS
3. New encounter → Fee Sheet → add a home E/M code → fee auto-populates at the charge-master rate, not the Medicare rate
4. Attempt to bill a psych code as an A1 clinician. **It should be blocked by the charge-ready gate**
5. Add an AWV plus a problem E/M on the same day with modifier 25. Both appear on the claim
6. Fees → Billing Manager → the encounter appears as a billable claim with Availity as its X12 partner. Do not transmit
7. Mark a TEST patient QMB. Attempt to generate a statement. **It should be blocked**
8. Administration → System → Logs → every action above appears, attributed correctly
9. RDS shows an encrypted database with backups; yesterday's EBS snapshot exists; the budget alarm is armed

Appendix B — Values worksheet

Keep with the password manager. Names of secrets, never contents.

VALUE	SOURCE
RDS endpoint	v2.3 Phase 2
Elastic IP	v2.3 Phase 3
EMR URL	emr.godwinsfamilycarellc.com
DB-MASTER / DB-OPENEMR / OE-ADMIN	Password manager
OAuth2 client_id and secret	AWS Secrets Manager, gfc/openemr/oauth-client
Key pair gfc-emr-key.pem	Break-glass only
GFC EIN	—
GFC Type 2 NPI, home care	1710678339
GFC Type 2 NPI, clinical	Pending
Availity Customer ID	Availity registration
PECOS login	Password manager

Sources

- 42 CFR 424.502 — institutional provider definition
- 42 CFR 424.520 — effective date of billing privileges
- 42 CFR 424.80 — assignment and reassignment of claims
- Consolidated CMS-855I/CMS-855R bulletin (CMS)
- Reassignment of Benefits (CMS)
- NPI enumeration and subpart guidance (CMS)
- CMS Place of Service code set
- Georgia nurse protocol agreement rules, effective 05/25/2026
- Georgia APRN Schedule II scope, O.C.G.A. § 43-34-25
- Georgia Composite Medical Board — nurse protocol agreements
- Medicare telehealth extended through 2027 (PYA)
- 2026 Medicare Parts A & B premiums and deductibles (CMS)
- Qualified Medicare Beneficiary Program (CMS)
- Medicare Annual Wellness Visit (CMS)
- OIG List of Excluded Individuals and Entities
- 42 CFR 410.73 — clinical social worker services
- NASW — Highlights of the 2026 Medicare Physician Fee Schedule Final Rule
- APA — CoCM and BHI FAQs, behavioral health care manager qualifications
- NPPES NPI Registry
- GFC_OpenEMR_Deploy_Setup_Guide_v2.3, July 22, 2026 (Part I, deployment, unchanged)